Undergraduate Medicine

Neurology Study Pack

CharaNas Medicine — Specialty Notes

Expanded notes with original schematic figures for revision. These are educational drawings inspired by standard undergraduate teaching themes, not copied textbook plates. Always follow your faculty curriculum and latest guidelines.

Section	Focus
1	Localization first
2	Stroke & thunderclap headache
3	Seizures, meningitis, raised ICP
4	Common outpatient syndromes
5	Checklist & books
6	Quick pearls from these notes
7	Recommended book sources

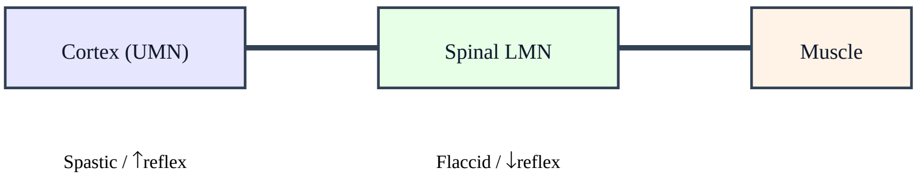
1. Localization first

Neurology begins with localization: cortex, subcortex, brainstem, spinal cord, root, plexus, peripheral nerve, neuromuscular junction, muscle.

UMN signs: spasticity, hyperreflexia, Babinski. LMN signs: flaccid weakness, hyporeflexia, fasciculations, atrophy.

Time course matters: hyperacute (stroke/SAH), acute (infection/inflammation), subacute, chronic progressive, relapsing–remitting.

Figure: UMN vs LMN lesion idea



Localize the lesion before naming the disease

Schematic figure for learning — verify details in your recommended textbooks.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about neurology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

2. Stroke & thunderclap headache

Stroke: sudden focal deficit — face/arm/speech pathways (FAST). Time is brain for thrombolysis/thrombectomy eligibility.

Differentiate ischemic vs hemorrhagic with urgent non-contrast CT. Glucose check is mandatory (hypoglycemia mimics stroke).

Thunderclap headache: peak intensity in seconds — exclude SAH with CT ± LP. Sentinel bleeds can precede catastrophic SAH.

Revision prompts

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3. Seizures, meningitis, raised ICP

First seizure workup: history (witness), glucose/electrolytes, consider CT/MRI and EEG as indicated. Status epilepticus is an emergency.

Meningitis: fever, neck stiffness, altered mentation — early antibiotics after cultures when bacterial disease suspected; do not delay for imaging if unsafe delay.

Raised ICP clues: headache worse lying down, vomiting, papilledema, focal signs — cautious LP only after risk assessment.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about neurology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

4. Common outpatient syndromes

Migraine: unilateral throbbing headache ± nausea, photo/phonophobia; red flags need exclusion (thunderclap, fever, focal neuro, cancer/immunosuppression, age >50 with new headache).

Neuropathy patterns: length-dependent sensory loss in diabetes; entrapment neuropathies (carpal tunnel) are common.

Parkinsonism: bradykinesia plus rest tremor/rigidity/postural instability — medication review for drug-induced causes.

Revision prompts

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- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

5. Checklist & books

Checklist: ABCs, glucose, focused neuro exam (mental status, cranial nerves, motor, sensory, reflexes, coordination, gait), imaging/LP as indicated.

Books: Adams and Victor; Harrison's neurology; local stroke pathway documents.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about neurology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

6. Quick pearls

High-yield reminders packaged with this specialty in CharaNas:

1. Stroke = sudden focal deficit — time is brain; use FAST/NIHSS pathways.

2. UMN vs LMN: spasticity/hyperreflexia vs flaccid/hyporeflexia/fasciculations.

3. SAH: thunderclap headache — urgent non-contrast CT ± LP.

4. Meningitis: fever, neck stiffness, altered mentation — early antibiotics.

5. Migraine: unilateral throbbing ± nausea/photo-phonophobia; rule out red flags.

- Link symptoms → anatomy/physiology → investigation → first management step.
- Write one OSCE-style explanation for a common presentation in this specialty.
- After reading, do the Short MCQ and Case-based Question for active recall.

7. Recommended book sources

Standard undergraduate references commonly used internationally. Use the edition your college recommends. Figures in commercial textbooks are copyrighted; use library/licensed access for full plates and photographs.

#	Reference
1	Adams and Victor's Principles of Neurology
2	Harrison's — Neurology
3	Clinical Neurology — Simon R. / Brazis

How to study with this PDF: read one section → sketch the figure from memory → answer the MCQs/cases → review mistakes → revisit the matching section.

Disclaimer: educational aid only; not a substitute for clinical guidelines, faculty teaching, or licensed textbooks.